

1. PATIENT PROFILE

Master registry demographics and baseline comorbidities

HF ID

7

CARE MR NO.

MR00007

Visit Type

☒ Inpatient

☐ Outpatient

☐ Home

PATIENT NAME

test 3

GENDER

Male

AGE & DATE OF BIRTH

55 years / 1970-10-29T18:30:00.000Z

PHONE NO.

9958769873

ADDRESS

—

Highest Education Level

☐ Primary

☐ Secondary

☐ Graduate

☒ Post Graduate

☐ None

Monthly Income

78000.00

Occupation

Retired

Caregiver Name

Caregiver Name

Relationship to Patient

wife

Caregiver Phone No.

9876543210

Insurance / Payment Mode

☒ Private Insurance

☐ Government Reimbursement

☐ Arogyasree

☐ Direct

Date of Visit / Hospitalization

dd-mm-yyyy

Date of Discharge

dd-mm-yyyy

Treating Cardiologist

Dr. K. Sridhar (Cardiologist)



Referring Doctor

Dr. Ananth Rao



Referred From (Department / Practice)

E.g. Cardiology Department / Community Clinic

Present Diagnosis

community clinic



2. INPATIENT DETAILS

Precipitating factors and admission context for heart failure hospitalizations

If admission for heart failure, please select precipitating factors for admission:

☐ Myocardial ischemia	☐ Atrial fibrillation
☒ Bradyarrhythmia	☐ Ventricular tachycardia
☐ Uncontrolled hypertension	☒ Infection
☐ Renal failure	☐ Anaemia
☐ Medication non adherence	☐ Excessive salt intake
☐ Excessive water ingestion	☐ Progressive worsening

Other Precipitating Factor

Excessive water ingestion

If admission for reasons other than heart failure, please specify reason(s):

trauma

No. of days hospitalized

3

Note: Update LOS & date of discharge upon receiving info.

3. INITIAL CLINICAL ASSESSMENT

Patient background, prior hospitalizations, vitals layout and diagnostic signs

Previous Diagnosis

Specify historical diagnostic parameters...

Medical History

☒ CABG	☒ PTCA
☒ Stroke	☒ Major Bleed
☒ Thrombolysis	
☒ Past MI	

No. of years ago

5

Location of MI

inferior

Others (Specify separate medical histories)

PVD

Recent Hospitalization(s)**History of hospitalization for heart failure:**☒ Yes☐ No**Date(s)**

feb 2020

Reason(s)

decompensated HF

VT/VF Risk Assessment☒ Documented episode of VT/VF☒ Complaints of Syncope / Pre-syncope**Frequency of episodes**

bigrmmy

☒ Documented PVCs**Number of PVCs**

2

Frequency / Pattern

bigrmmy

☒ Documented NSVT**Frequency of episodes**

4

Vitals Metrics

Weight (kg) *

70.00

☐ Unable to weigh (Measure at earliest opportunity)

Height (Cm)

170.00

CALCULATED BMI

24.2

Heart Rate (resting) (Bpm) *

72

Variability

☒ Regular

☐ Irregular

Respiratory Rate

18

O₂ Saturation (%)

98.00

Blood Pressure: Sitting / Supine (mmHg)

120

/

80

Blood Pressure: Standing (mmHg)

115

/

76

Mental Status

☐ Alert / Oriented

☒ Confused

☐ Drowsy

Symptoms

Dyspnea at rest:

☒ Yes

☐ No

Weight Loss:

☒ Yes

☐ No

Dyspnea with exertion:

☒ Yes

☐ No

Syncopy:

☒ Yes

☐ No

Fatigue:

☒ Yes

☐ No

PND:

☒ Yes

☐ No

Orthopnea:

☒ Yes

☐ No

Muscle Cramps:

☒ Yes

☐ No

Loss of appetite/Bloating:

☒ Yes

☐ No

Wheeze:

☒ Yes

☐ No

Decreased exercise tolerance:

☒ Yes

☐ No

Giddiness:

☒ Yes

☐ No

Weight Gain:

☒ Yes

☐ No

Other:

☒ Yes

☐ No

sympton

Clinical Signs of Volume Overload

Peripheral edema:

☒ Yes

☐ No

Ascites:

☒ Yes

☐ No

Rales:

☒ Yes

☐ No

Jugular venous pressure:

☒ Yes

☐ No

Hepatomegaly:

☒ Yes

☐ No

Other:

☒ Yes

☐ No

cold

4. FINAL CLINICAL ASSESSMENT

Discharge classification, structural etiologies, and outcomes layout

Type of Heart Failure *

☐ HFrEF (HF with reduced EF)

☐ HFpEF (HF with preserved EF)

HF Etiology

Cardiovascular

☒ Ischemic

☒ Toxic (Alcohol, Cocaine, Chemotherapeutic)

☒ Idiopathic (Dilated)

☒ Hypertrophic

☒ Restrictive (Amyloid, Sarcoid)

☒ Valvular/ Rheumatic heart disease

☒ Hypertensive

☒ Tachycardia Induced

☐ Metabolic Diseases/Hemochromatosis/Wilson's Disease

Non-cardiac

☒ Pregnancy

☒ Thyroid Disease

☒ Pheochromocytoma

☒ Chronic Renal Disease

Pulmonary

☒ Cor Pulmonale secondary to COPD

☒ Pulmonary Hypertension

Others (please specify):

Specify other etiology...

☒ HIV and Viral Cardiomyopathy

☒ Inflammatory cardiomyopathy

☒ Reduced EF with/wo previous MI

Comorbidities & Risk Factors

Comorbidities

☒ Associated CAD

☒ Anemia

☒ Renal Failure

☒ CVA

☒ Diabetes Mellitus

☒ Severe musculoskeletal disease

☒ Hypertension

☒ Cancer

☒ Valvular Disease

☒ APD

☒ Asthma

☒ Bleeding diathesis

☒ COPD

☒ PVD

☒ OSA

Others:

Excessive water ingestion

Risk Factors

☒ Smoking

☒ Alcohol

Others:

Excessive water ingestion

Stage of HF *

☐ Stage A

☐ Stage B

☒ Stage C

☐ Stage D

Functional Status *required *

☐ NYHA Class I

☐ NYHA Class II

☐ NYHA Class III

☐ NYHA Class IV

AF Status *required *

☒ Permanent

☐ Paroxysmal

☐ Persistent

☐ NSR

Major Adverse Cardiac Events

☒ Hospitalization

☒ Stroke

☒ Major Bleed

☒ Severe Arrhythmia

☒ Major Procedures:

☒ Other:

Excessive water ingestion

☒ **Death:**

Date:

dd-mm-yyyy

Location:

☒ Home ☐ Hospital

Reason:

Excessive water ingestion

Final Clinical Notes / Summary

Excessive water ingestion

5. INVESTIGATIONS

Complete clinical test records, laboratory metrics, and physiological studies grid mapping

ECG *required

Date of test:

dd-mm-yyyy

 QRS duration:

5.00

Rhythm

☐ Sinus

☐ AF

☐ Other:

AV Conduction

☐ Normal

☐ 1st degree AV block

☐ 2nd degree AV block

Q Waves

☒ Yes

Leads:

Excessive water ingestion

3rd degree AV block

None

Blockages

LBBB

RBB

Excessive water ingestion

Extra Beats

APC

VPC

None

QT Interval

QT: 0.00

QTC: 0.00

Chest X-ray

Date of test: dd-mm-yyyy

CT ratio: 0.00

Pleural effusion

Others: CT

PVH

Pulmonary edema

ECHO

Date of test: dd-mm-yyyy

EF%: 0.00

E/A ratio: 0.00

RV TAPSV: 0.00

E/E' ratio: 0.00

E deceleration time: 0.00

Left Atrium Dimension

Left Ventricle Systole

Left Ventricle Diastole

MR mitral regurgitation:

None

1plus

2plus

3plus

4plus

Other Valves: hello

RV Systolic Pressure: hello

RV Function:

Normal

Impaired

RWMI:

None

Global

Inferior

Anterior

Lateral

Holter/ Event Recorder

Date of test: dd-mm-yyyy

VPC:

Ventricular Arrhythmia:

No

Yes

Complex VPC

NSVT

VT

Atrial Arrhythmias:

None

APCs

AF

Heart rate variability: hello

Stress Test

Date of test: dd-mm-yyyy

Not Done

Done

localhost:3000/hf-form/view/3

10/19

▪ METS achieved:

0.00

▪ Target heart rate achieved:

Anterior

▪ Ischemic changes:

☒ Yes

☐ No

▪ Arrhythmias:

☐ Yes

☒ No

MRI

☒ LVEF:

0.00

☐ Scar Present

☐ Scar Absent

Date of test:

dd-mm-yyyy

PET

Date of test:

dd-mm-yyyy

6-Minute Walk Test

Date of test:

dd-mm-yyyy

☒ Done:

▪ Distance walked in m:

0.00

▪ Heart rate recovery in first 1 minute:

Anterior

☐ Not Done, Reasons:

Anaerobic Threshold

Date of test:

dd-mm-yyyy

Angiogram

Date of test:

dd-mm-yyyy

☒ Done:

☐ Normal

☐ 1 vessel disease

☐ 2 vessel disease

☐ 3 vessel disease

☐ LMCA

☐ Not Done

Endomyocardial biopsy

Date of test:

dd-mm-yyyy

☒ Done

☐ Not Done

Vaccinations

☒ Pneumococcal (Date of test:

dd-mm-yyyy

☒ Influenza (Date of test:

dd-mm-yyyy

Blood group

☐ A+

☐ A-

☐ B+

☐ B-

☐ AB+

☐ AB-

☐ O+

☐ O-

Lab Tests

(*Potassium and Creatinine tests are required for all follow-up visits)

PLEASE SPECIFY:	RESULTS	DATE	WHEN REQUIRED:	RESULTS	DATE
☒ Potassium*	4.5	dd-mm-yyyy	☐ BNP		dd-mm-yyyy

☒ Creatinine*	4.5	dd-mm-yyyy	☐ NT-pro BNP		dd-mm-yyyy
☒ Hb	4.5	dd-mm-yyyy	☐ LDL		dd-mm-yyyy
☒ Calcium	4.5	dd-mm-yyyy	☒ INR	4.5	dd-mm-yyyy
☒ BUN	4.5	dd-mm-yyyy	☐ ST2		dd-mm-yyyy
☒ Glucose	4.5	dd-mm-yyyy	☐ Other:	4.55555	dd-mm-yyyy
☒ HBa1c	4.5	dd-mm-yyyy			
☒ Magnesium	4.5	dd-mm-yyyy			
☒ Sodium	4.5	dd-mm-yyyy			
☒ TSH	4.5	dd-mm-yyyy			
☒ T3	4.5	dd-mm-yyyy			
☐ T4		dd-mm-yyyy			

6. MEDICAL THERAPY (DOSE & FREQUENCY)

Guideline-directed medical therapy with dosing details

RECOMMENDED CONSULTS

4.5

DRUG INTOLERANCE(S) / CONTRAINDICATIONS

4.5

BETA-BLOCKER

☒ Carvedilol

6 /per day

☒ Bisoprolol

4 /per day

☒ Metoprolol Succinate

5 /per day

☐ Nebivolol

☐ Other:

CONTRAINDICATION/REASON NOT USED:

☒ Bradycardia

☒ Heart blocks

☒ COPD/ Asthma

☒ Hypotension

☒ Other

4.5

ACE INHIBITOR

☒ Enalapril

/per day

☒ Ramipril

/per day

☒ Lisinopril

/per day

☒ Perindopril

/per day

☒ Other:

Varshitha Appam

/per day

CONTRAINDICATION/REASON NOT USED:

☒ Elevated creatinine (Renal failure)

☐ Cough

☐ Other

☐ Hyperkalemia

☐ Hypotension

ANGIOTENSIN RECEPTOR BLOCKER

☒ Valsartan

/per day

☒ Losartan

/per day

☒ Telmisartan

/per day

☒ Olmesartan

/per day

☒ Other:

Varshitha Appam

/per day

CONTRAINDICATION/REASON NOT USED:

☒ Elevated creatinine (Renal failure)

☒ Hypotension

☒ Hyperkalemia

☒ Others

ALDOSTERONE ANTAGONIST

☒ Spironolactone

/per day

☒ Eplerenone

/per day

CONTRAINDICATION/REASON NOT USED:

☒ Hyperkalemia

☒ Elevated creatinine (Renal failure)

☒ Hyponatremia

☒ Other

HYDRALAZINE

☒

/per day

NITRATE

☒

hello

5

/per day

☒

hello

6

/per day

ANTICOAGULATION

☒ Warfarin

INR:

34

Target INR:

35

☒ Vitamin K Inhibitor

hello

5

/per day

☒ NOAC

Name

Dose

/per day

☒ Acitrom

Dose

/day

☐ UFH

☐ LMWH

ANTI-PLATELET

☒ Aspirin

Dose

/per day

☒ Clopidogrel

76

/per day

☒ Prasugrel

Dose

/per day

☒ Ticagrelor

Dose

/per day

ANTIARRHYTHMIC

☒ Amiodarone

4

/per day

☒ Other:

hello

5

/per day

DIURETIC

☒ Furosemide

e

/per day

☒ Torsemide

Dose

/per day

☒ Metolazone

Dose

/per day

☒ Other:

hello

5

/per day

CONTRAINDICATION/REASON NOT USED:

☒ Hyponatremia

☒ Hypokalemia

☒ Worsening renal failure

☒ Hypotension

☒ Other

DIGOXIN

☒ /per day

IVABRADINE

☒ Ivabradine /per day

STATINS

☒ Atorvastatin /per day

☒ Simvastatin /per day

☒ Rosuvastatin /per day

ANTIDIABETICS

☒ Sulfonylureas /per day

☒ Metformin /per day

☒ Glitazone /per day

☒ Gliptin /per day

☒ Acarbose Derivative /per day

☒ Human Insulin /per day

☒ Synthetic Insulin /per day

OTHER

☒ Anti-hypertensive /per day

☒ Thyroxine /per day

☒ /per day

☒ /per day

☐

\n\n

7. DEVICE THERAPY

Current implanted devices and eligibility assessment

CURRENT DEVICE THERAPY

☐ No Device

☒ Yes, Has Device

☒ CRT-P

☒ CRT-D

☒ ICD-SC

☒ ICD-DC

☒ Dual Chamber Pacer Mode

☒ Single Chamber Pacer Mode

☒ Other

hello

Device Brand / Model

hello

ELIGIBILITY FOR DEVICE THERAPY

☐ Not Eligible

☒ Eligible

☒ CRT-P

☒ CRT-D

☒ ICD-SC

☒ ICD-DC

☒ Dual Chamber Pacer Mode

☒ Single Chamber Pacer Mode

☒ Other

hello

Recommended Brand / Model

hello

Patient Acceptance

☐ Yes

☒ No

If No, reasons:

hello

DATE OF IMPLANT

10-11-2001

ICD THERAPY ADMINISTERED

☒ ICD Shock

of shocks

5

of appropriate shocks

5

of inappropriate shocks

4

Cause of Shocks

hello

☒ ATP

of times

8

ATP successful?

☐ Always ☒ Most times ☐ Sometimes ☐ Not successful

TECHNICAL LOG PARAMETERS

BiV pacing (%)

98.00

AFib burden

2

NSVT episodes (#)

32

SVT episodes (#)

23

Device Volume alert

hello

NOTES

hello

8. PATIENT EDUCATION

Counseling topics documented for this visit

RECOMMEND PATIENT EDUCATION

☒ Diet: 2000-mg salt restriction

☒ Exercise/activity promoted

☒ Daily weight monitoring

☒ Disease process explained

☒ Smoking cessation

- ☒ Alcohol cessation
- ☒ Medication compliance
- ☒ What to do for worsened symptoms
- ☒ Device therapy education
- ☒ Other

hello

9. RECOMMENDATIONS

Discharge plan, follow-up and therapeutic guidance

☒ FLUID AND DIET

hello

☒ EXERCISE

Yes

☒ YOGA

hello

☒ SMOKING CESSATION

Details for Smoking Cessation...

☒ STRESS MANAGEMENT

Details for Stress Management...

☒ **DRUGS**

hello

☒ **INVESTIGATIONS**

Yes

☒ **PROCEDURES**

Yes

☒ **OTHERS**

Details for Others...